

Misallocated Care: Needs, Power, and Responsibility in Public Elderly Care

A Five-Layer and Sustainable-Capacity Diagnostic Model

Author / project participant: Sulky

Project version: Project Aletheia v0.4.0

Date: 2026-09-17

DOI for Project Aletheia v0.4.0: 10.5281/zenodo.22766372 (<https://doi.org/10.5281/zenodo.22766372>)

Repository: bbbzzzzzzZ/Project-Aletheia (<https://github.com/bbbzzzzzzZ/Project-Aletheia>)

Status: Working paper draft; not peer reviewed; not an implemented policy proposal.

Recommended SSRN areas: Social & Political Philosophy; Philosophy of Law; Political Theory; Political Economy; Public Policy; Health Policy; Social Welfare; Aging and Long-Term Care.

Abstract

This working paper develops a diagnostic framework for public care by combining four elements from Project Aletheia: a five-layer model of needs, a need-power-responsibility theory, a responsibility white-box model, and a sustainable-capacity test. The subject-consistency component is abbreviated as NPR-SC: Need-Power-Responsibility Subject Consistency. The central claim is that many institutional failures are not only failures of resource supply, but failures of subject alignment and capacity disclosure. A system may recognize a need, assign authority to one actor, shift responsibility to another, and leave the actual bearer of cost without corresponding power, resources, recognition, or exit.

Elderly care is used as the primary case. The paper argues that elderly care is not a single type of demand. It spans life and safety needs, basic autonomy, public operation needs, externally collaborative development needs, and voluntary consumption. The institutional problem arises when first-layer life and safety needs and third-layer public operation needs are pushed into fifth-layer market consumption logic, or when public commitments remain formally in place while the sustainable capacity to meet them is absent, concealed, or depleted over time. In such a structure, older persons may be treated as consumers if they can pay, families as residual caregivers if public provision is insufficient, and care workers as low-status labor despite carrying the practical burden of a public need. The French Orpea scandal, the English NHS/social care discharge problem, and China's long-term care and family-care pressure are used as illustrative diagnostic cases, not as exhaustive empirical studies (Castanet 2022; IGAS and IGF 2022; CQC 2024; King's Fund 2025; Ministry of Civil Affairs and China National Working Commission on Aging 2026).

The paper does not propose a complete pension or long-term care policy. It offers a conceptual test: who has the need, who receives authority, who bears responsibility, who pays the cost, who gains from the arrangement, who can correct it when the chain breaks, and whether the system can continue without consuming those who cannot realistically exit. The elderly care case shows how public systems can appear to increase supply while leaving subject consistency unrepaired. The paper concludes that public care should be evaluated not only by beds, facilities, market scale, or service volume, but by whether life-safety needs are protected as a public floor, whether care labor is recognized as a public operation function, whether capacity gaps are honestly disclosed, and whether the people who bear responsibility receive corresponding authority, resources, institutional protection, and correction

channels.

Keywords: elderly care; long-term care; needs; responsibility; public authority; social welfare; care work; political philosophy; institutional design; Project Aletheia.

Chinese summary

本文将 Project Aletheia 的五层需求模型、需求—权力—责任理论、“责任白箱”模型和可持续能力测试结合起来，用社会养老问题作为案例，说明很多制度失败并不只是资源不足，而是需求、权力与责任的主体链条被拆开，同时公共能力缺口被隐藏或转嫁。养老需求并非单一需求：失能老人的基本照护属于第一层生命安全需求，健康老人的生活自主属于第二层基本自由需求，长期护理保险、社区设施和护理员培养属于第三层公共运行需求，认知症干预、临终关怀等专业服务属于第四层外部协作型发展需求，高端养老社区和银发消费则属于第五层自主消费需求。

当前养老问题的核心断裂在于：第一层和第三层需求被大量推入第五层市场消费逻辑，或者公共系统在能力不足时仍维持形式承诺，最后由没有现实退出权的老人、家庭和照护劳动者硬扛缺口。老人只有在有支付能力时才像消费者一样获得服务；家庭在公共供给不足时成为剩余照护者；护理员承担了实际公共责任，却缺乏相称的收入、权力、职业承认和制度保护。本文主张，养老制度不能只用床位数、机构数、市场规模或服务量评价，而应追问主体一致性和可持续能力是否恢复：老人的生命安全需求是否被无条件保底，护理员是否获得与承担相匹配的权力和资源，家庭是否不再被迫成为唯一的次级责任者，公共能力缺口是否被真实公开。

Note on evidence and scope

This is a conceptual working paper. The elderly care case is used as a diagnostic illustration, not as a completed empirical study. Population aging, long-term care insurance, disabled older persons, and care worker shortages are discussed with reference to current publicly available estimates and project materials. Before formal SSRN submission, all numerical claims should be checked against primary or authoritative sources and dated explicitly.

The argument does not deny that resource scarcity matters. It argues that scarcity alone cannot explain the institutional structure of care. A society may lack money, labor, facilities, or administrative capacity; but it may also misclassify needs, transfer public responsibility to families or markets, conceal capacity gaps, and devalue the workers whose labor sustains the system. These are different failures and require different diagnoses. The paper therefore distinguishes honest scarcity from responsibility displacement: a real capacity shortage should be disclosed, negotiated, and corrected, not moralized into an obligation carried silently by older persons, families, or care workers.

1. Introduction: when public care is treated as private consumption

Elderly care is often described as a demographic challenge, a fiscal challenge, a family challenge, or a market opportunity. Each description is partly true. Aging populations increase care needs. Long-term care financing is expensive. Families often remain the first practical caregivers. Private providers may expand service capacity. Yet these descriptions can hide the deeper institutional question: what kind of need is elderly care, and who should bear responsibility for it?

If an older person needs help eating, bathing, moving, taking medication, avoiding injury, or surviving cognitive decline, the matter cannot be reduced to consumer preference. It is not like choosing a larger apartment, a better hotel, or a luxury retirement community. At least part of elderly care belongs to the conditions of life and safety. Another part belongs to public operation: the training, financing, staffing,

supervision, and continuity of care services that make survival and dignity possible at scale.

The difficulty is that modern care systems often blur these layers. A policy may speak of a basic public guarantee, but the practical burden may still fall on families. A market may expand high-end elderly services, but low-income or disabled older persons may remain under-served. A government may purchase services rather than directly provide them, but the workers who perform care may remain low-paid, overburdened, and socially devalued. Facilities may increase in number, while the person who needs care has little voice in the rules that govern their life. The result is not a simple absence of supply. It is a misallocation of need, power, and responsibility.

Project Aletheia proposes a diagnostic vocabulary for this problem. It asks: whose need is being recognized? What kind of need is it? Who receives power in order to respond to it? Who is made responsible for cost, labor, failure, and repair? Is the responsibility subject the same as the need subject, or has responsibility been shifted onto someone else? Do those who bear practical responsibility receive adequate resources, authority, recognition, and protection? If not, the institution may appear to “handle” a social problem while leaving the underlying subject chain broken.

This paper calls that diagnostic standard NPR-SC: Need-Power-Responsibility Subject Consistency. The phrase is intentionally technical, but the question is simple:

Is the person or community whose need is being addressed also properly connected to the power and responsibility created in response to that need?

Put less technically, NPR-SC is a needs-first and responsibility-closing theory of distributive justice. It begins not from property, contract, utility, or preference alone, but from the classification of needs and the question of whether the responsibilities created by those needs are returned to the proper subject with adequate authority and capacity. Its purpose is not to abolish markets or organizations, but to prevent public needs from being processed through arrangements that sever need, power, and responsibility.

To avoid unnecessary conceptual inflation, the paper has three primary analytic components: NPR-SC, the responsibility white-box, and the sustainable-capacity test. Ethical tethering and dependency-based value inversion are secondary mechanisms identified inside the elderly care case. They help explain how a responsibility chain becomes psychologically and culturally stabilized, but they are not separate theories competing with the main framework.

When the answer is no, institutions generate domination, invisibility, and moral inversion. Older persons may become passive recipients of services organized without them. Families may become the hidden shock absorbers of public under-provision. Care workers may become the practical carriers of a public responsibility while being treated as low-status labor. The market may gain power to organize care without bearing the full public responsibility for the life-safety needs it administers.

The elderly care case is therefore not merely a policy example. It is a test of whether a political and moral theory can distinguish care as a public condition from care as a commodity.

The paper is not a country-specific actuarial report on pension finance or long-term care insurance. It proposes a diagnostic framework that can travel across different institutional settings: for-profit nursing home crises in France, the hospital-social-care interface in England, and family-centered elder care pressures in East Asian societies. These systems differ greatly, but they can display a similar pattern when life-safety care, public operation capacity, and responsibility-bearing subjects are pulled apart.

2. The five-layer model of needs

Aletheia distinguishes five layers of needs. The model is not a complete psychology of desire. It is a public reasoning tool: it helps decide whether a claim should be guaranteed, protected, coordinated, supported through external resources, or left to voluntary consumption.

2.1 Layer one: life and safety needs

The first layer contains needs without which the person's basic life and safety are at risk: food, water, minimal shelter, emergency medical care, essential hygiene, basic protection from violence and severe neglect, and the minimal conditions needed to remain alive and physically safe.

In elderly care, this layer includes basic care for disabled older persons: eating, cleaning, turning over, preventing bedsores, medication safety, emergency response, and protection from abandonment or abuse. These needs should not create a personal debt of dignity. A person does not become morally inferior because they need help eating or bathing. Nor does the appearance of such need mean that the family alone automatically becomes the first and final bearer of responsibility.

2.2 Layer two: basic freedom and independent development needs

The second layer concerns basic freedom and independent development: the private space to think, learn, communicate, create, associate, and arrange one's life without unnecessary interference. It is a defensive layer. It does not require the public to fund every aspiration, but it requires others not to block ordinary self-direction when no public burden or harm is created.

For older persons, this includes the freedom to maintain social life, learn, create, choose daily rhythms, preserve relationships, and avoid being treated merely as a medical or administrative object. A healthy older person's life should not be reduced to passive dependence simply because they are old.

2.3 Layer three: public operation needs

The third layer concerns public operation: infrastructures and organized functions that keep shared life possible. These include public health, emergency response, water, electricity, basic logistics, public food systems, environmental management, judicial systems, education, and care infrastructures.

In elderly care, long-term care insurance, community care centers, public assessment systems, training pipelines for care workers, regulatory supervision, emergency support, and care continuity belong here. These are not private luxuries. They are part of the background system through which a society handles a predictable human condition: aging, disability, dependency, and death.

2.4 Layer four: externally collaborative development needs

The fourth layer contains needs that require external resources, professional support, specialized facilities, cross-regional coordination, or public allocation beyond what an individual can provide alone. They are not automatically unconditional guarantees in the same way as layer one, but they may deserve public support when they are necessary, feasible, safe, and publicly valuable.

In elderly care, specialized dementia intervention, rehabilitation, hospice care, assistive technologies, complex medical-social coordination, and advanced care planning may fall into this layer. The relevant question is not whether such services are desirable; many are. The question is what procedure fairly determines access when resources are limited.

2.5 Layer five: voluntary consumption and non-essential preference

The fifth layer concerns voluntary consumption, luxury, comfort, status, aesthetic preference, and optional enjoyment. It includes high-end retirement communities, premium rooms, travel-based retirement, luxury “silver economy” products, and other services that may be legitimate but should not consume the resources needed to protect the first four layers.

This is not an anti-market claim. Aletheia does not deny that people may pursue comfort, beauty, convenience, and luxury. The constraint is that layer-five expansion must not devour the public capacity required for life-safety needs, basic freedom, public operation, and fair access to necessary external collaboration.

Figure 1. Simplified five-layer need model

The five layers can be summarized as follows:

- Layer 5 Voluntary consumption and non-essential preference
Luxury, comfort, status, optional market demand
Rule: legitimate only if it does not consume Layers 1-4
- Layer 4 Externally collaborative development needs
Specialized care, advanced support, equipment, cross-system resources
Rule: allocated by necessity, feasibility, safety, and public value
- Layer 3 Public operation needs
Infrastructure, staffing, training, insurance, regulation, continuity
Rule: collective public responsibility; delegation must carry capacity
- Layer 2 Basic freedom and independent development needs
Thought, learning, creation, association, private autonomy
Rule: do not block unless harm or public burden is created
- Layer 1 Life and safety needs
Survival, emergency care, essential hygiene, protection from neglect
Rule: unconditional public floor; no dignity debt

The central warning is downward displacement and upward capture. Downward displacement occurs when public or institutional responsibility is pushed onto individuals, families, or low-power workers. Upward capture occurs when higher-layer consumption or institutional interest consumes the resources required to protect lower-layer needs.

3. Elderly care as a multi-layer demand

Elderly care becomes difficult to classify because it spans all five layers at once.

Layer	Elderly care example	Institutional meaning
Life and safety	Basic care for disabled older persons, feeding, hygiene, medication safety, emergency response	Public floor; should not depend solely on family wealth
Basic freedom	Social life, learning, daily autonomy, self-arranged activity	Defensive right; should not be unnecessarily blocked
Public operation	Long-term care insurance, community care facilities, care worker training, supervision	Collective public responsibility organized through public systems
External collaboration	Dementia intervention, hospice, rehabilitation, assistive technologies	Procedural allocation and professional support

Layer	Elderly care example	Institutional meaning
Voluntary consumption	High-end retirement communities, luxury services, travel retirement	Legitimate consumption, but lower priority than layers one to four

The central diagnostic claim is this:

The crisis of elderly care emerges when first-layer and third-layer needs are processed through fifth-layer logic.

When life-safety care is treated as consumption, access becomes conditional on purchasing power. When care infrastructure is treated mainly as a market opportunity, the public system may become a purchaser, regulator, or subsidy distributor while withdrawing from direct responsibility for capacity, labor standards, and service continuity. When family care is presumed as the default, the cost of public under-provision is hidden inside households. When care labor is treated as low-status labor, the society receives essential work while refusing to recognize its dependence on the worker.

This is why elderly care cannot be evaluated only by asking whether service supply has increased. Supply may increase while the wrong layer is being expanded. A society may build high-end facilities while basic care remains fragile. It may encourage market participation while care workers remain underpaid. It may promote “silver economy” growth while families continue to absorb the daily consequences of disability. The question is not whether something is being supplied, but whether the right kind of need is being answered by the right kind of responsibility.

4. Need, power, and responsibility

Aletheia’s need-power-responsibility theory begins from a simple proposition: a need does not automatically create authority. A need must first be classified by its subject, layer, public implications, and cost structure.

If I want a luxury service, that is primarily my private demand. Others may voluntarily provide it, and markets may lawfully offer it, but I do not thereby acquire a claim on public resources or other persons’ labor. If a hospital needs night-shift staffing, that is not the private moral debt of individual doctors; it is an organizational and public operation need requiring staffing, compensation, training, rotation, and backup. If a disabled older person needs help eating, that need appears through one person, but the underlying vulnerability belongs to everyone: any person can age, become disabled, lose family support, or require care. In that sense, basic elderly care is not a private accident. It is a common vulnerability manifested in a particular life.

This distinction leads to the principle of responsibility by need-subject:

- Private needs generate first responsibility in the person whose private demand it is.
- Public needs generate first responsibility in the public as the community to whom the need belongs.
- Organizational needs generate responsibility in the organization or task system that creates them.
- Role needs generate responsibility in the role system that defines, trains, assigns, and supports the role.
- Harmful needs do not enter the satisfaction pathway; they enter limitation, refusal, or correction.

Here public responsibility must be distinguished from state responsibility. In many welfare-state discussions, to call elderly care a public responsibility means that the state, public finance, or public administration has duties of funding, regulation, subsidy, or service purchase. Aletheia uses the term more broadly. Public responsibility means collective public responsibility: the responsibility belongs first

to the community whose members share the underlying vulnerability, while the state is a delegated institutional agent for organizing that responsibility.

The government is therefore not identical with public responsibility. It is an authorized organizer, coordinator, and executor of a responsibility that belongs to the public community. Markets, care institutions, professional organizations, families, and individuals may all participate in execution or support, but their participation does not change the final location of the responsibility. Nor can the government claim that public responsibility has been completed merely because it has contracted the task out. Delegation may change the execution subject; it does not change the responsibility subject.

In elderly care, this means that the first responsibility-subject is not the state alone, nor the older person's family, nor the care worker, nor the nursing home. It is the public community as a community of persons who may age, become disabled, lose family support, or require care. The state is the institutional proxy through which this collective responsibility is organized. Care institutions and workers are execution subjects. Families are participants and relational supporters. Older persons are the visible need-subjects through whom a common vulnerability becomes concrete.

In elderly care, the problem is that these categories are often crossed. The older person's life-safety need is treated as a family expense. The public operation need for trained care workers is treated as a market recruitment problem. The organization's need for cheap labor is sometimes disguised as the worker's personal calling or moral duty. The family's exhaustion is interpreted as lack of filial virtue rather than as evidence that the responsibility chain is overloaded.

Once the need is misclassified, power and responsibility follow the wrong path. Markets receive organizing power. Families receive residual responsibility. Workers receive physical and emotional burden. Older persons receive conditional service. The public receives reassurance that "the sector is developing," while the subject chain remains broken.

5. The responsibility white-box

The responsibility white-box is a method for opening the black box of institutional responsibility. Instead of asking "who is responsible?" as if responsibility were a single object, it asks several questions:

1. Who experiences the need?
2. Who interprets the need?
3. To whom does the need ultimately belong?
4. Who is authorized to respond?
5. Who supplies labor?
6. Who pays the cost?
7. Who gains revenue, status, or control from the arrangement?
8. Who bears failure when the arrangement breaks?
9. Who can refuse, appeal, exit, or demand correction?
10. Who becomes invisible in the official account?

Applied to elderly care, the white-box reveals at least four subject positions.

First, the older person is the need-subject. The person's bodily vulnerability, dignity, safety, autonomy, and fear are the reason the care system exists. Yet older persons often have limited influence over institutional rules, pricing, staffing, grievance procedures, or daily care arrangements.

Second, the family is often the residual responsibility-subject. When public provision is insufficient and market care is expensive, families absorb time, money, emotional stress, and career disruption. This is often described through moral language: filial piety, family duty, or intergenerational affection. These values matter, but they can also hide forced substitution. A family that loves an older person may still be crushed by tasks that should have been publicly organized.

Third, the care worker is the practical execution-subject. Care workers lift bodies, clean waste, manage risk, calm fear, absorb anger, observe deterioration, and protect safety. Yet the worker may have little authority over staffing ratios, care plans, institutional priorities, wages, or safety protocols. The worker is responsible in practice but weak in institutional power.

Fourth, the public system is the proper collective responsibility-subject for the first-layer and third-layer components of elderly care. If everyone may become old, disabled, or dependent, then the public cannot treat basic care as someone else's private problem. The public system does not have to personally perform every care task, but it must remain responsible for the floor, the capacity, the labor conditions, the accountability structure, and the correction of failure.

The white-box diagnosis is therefore:

The need-subject, authority-subject, cost-subject, labor-subject, and correction-subject are separated. The institution appears to respond to care while distributing power and burden unevenly.

6. NPR-SC: subject consistency as an evaluative standard

NPR-SC stands for Need-Power-Responsibility Subject Consistency. A system has subject consistency when the subject whose need justifies authority is properly connected to the responsibility and accountability created by that authority. It does not require that one person or institution do everything. Complex societies need division of labor. But it requires that division not become displacement.

In elderly care, the ideal form can be expressed as:

Older persons' life-safety needs → public floor and care infrastructure → public responsibility with supported execution.

The distorted form is:

Older persons' life-safety needs → marketized service power → family and care worker burden.

Another ideal form is:

Public need for long-term care capacity → public authority to organize training, financing, staffing, and oversight → public responsibility for labor conditions and service continuity.

The distorted form is:

Public need for care capacity → private providers and fragmented labor markets → low-status workers absorb the operational burden.

NPR-SC does not ask whether markets are present. Markets may provide useful services. It asks whether market power is being used to process a need whose underlying responsibility remains public. If so, the public system must not disappear behind the contract. But this cannot mean only that the public system keeps minimum service rules, labor rules, transparency requirements, grievance channels, and formal protections against exclusion.

The existence of rules is not the same as the production of adequate care. A minimum standard may be lower than real need. A regulator may lack information, staffing, or enforcement capacity. OECD's work on long-term care emphasizes that working-condition reforms depend not only on the existence of staffing requirements or transparency rules, but on the level of standards, financing, compliance, enforcement, training, wages, and social dialogue (OECD 2023). The U.S. Government Accountability Office likewise found oversight gaps in nursing home abuse reporting and referral systems despite federal quality standards and state survey processes (U.S. GAO 2019). A provider may remain technically compliant while operating with low wages, high turnover, thin staffing, short care time, and exhausted workers. A system may add a higher service requirement without adding the money, personnel, training time, or social conditions needed to meet it, thereby worsening the burden on the very workers who are supposed to deliver better care.

For that reason, NPR-SC distinguishes two public responsibilities after delegation.

First, there is compliance responsibility: preventing abuse, fraud, exclusion, concealment, and denial of complaint channels. This is the responsibility to ensure that the delegated actor does not violate basic rules.

Second, there is capacity-maintenance responsibility: ensuring that the delegated public task has the real conditions required for sustainable completion. This includes asking how many workers, how much funding, how much time, what level of training, what retention conditions, and what public support are needed to meet the stated standard. If these conditions do not exist, the remaining gap is still a public problem. It cannot be declared solved merely because a contract has been signed or a formal standard exists.

The deeper test is therefore not only “is the provider compliant?” but “does the system jointly maintain the real conditions under which the public task can be done well without consuming non-exitable subjects?”

Subject consistency also does not erase family responsibility. Families may love, accompany, decide, support, and care. But when a predictable public vulnerability is pushed onto families without adequate support, family responsibility becomes a mask for public withdrawal. The family then becomes a secondary responsibility-subject used to absorb a primary public need.

7. From current capacity to sustainable capacity

NPR-SC identifies whether need, power, and responsibility remain connected. But elderly care also requires a capacity test. A system may have a correct moral classification and still fail because it lacks money, workers, facilities, management capacity, or social trust. The problem is not that scarcity exists. The problem is when scarcity is denied, hidden, or transferred onto people who cannot realistically refuse.

For this reason, Aletheia distinguishes current capacity from sustainable capacity.

Current capacity can be described as:

$$C_{\text{actual}} = f(A, M, E)$$

where:

- A means authority: the legal and institutional power to define eligibility, allocate resources, regulate providers, enforce standards, and correct failure.

- M means material resources: fiscal resources, facilities, beds, equipment, staff, training capacity, time, and administrative infrastructure.
- E means execution: the practical ability to deliver services continuously, safely, and reliably through actual organizations and workers.

This first test asks whether a system can operate now. But elderly care is not a one-time project. It is a long-duration system exposed to demographic aging, worker fatigue, household depletion, fiscal pressure, social stigma, and changing expectations. Therefore a second test is needed:

$$C_{\text{sustainable}} = f(A, M, E, T, F)$$

where:

- T means time: whether the arrangement can survive demographic change, accumulated exhaustion, staff turnover, family burnout, and intergenerational burden.
- F means feedback: whether social culture, public recognition, grievance channels, institutional learning, and policy correction can reveal and repair failure instead of normalizing it.

These are heuristic notations, not mathematical functions in the strict sense. They are used to keep the diagnostic sequence visible: authority, resources, execution, time, and feedback must be examined together. At this stage of the theory, the variables are best understood as ordinal and comparative indicators rather than quantified parameters.

Component	Observable questions
A: authority	Who can define eligibility, allocate resources, enforce rules, change standards, and correct failure?
M: material resources	Are there enough funds, facilities, beds, equipment, supplies, and paid work hours?
E: execution	Can actual organizations and workers deliver the promised care safely and continuously?
T: time	Can the arrangement survive aging, turnover, burnout, family depletion, and intergenerational pressure?
F: feedback	Can residents, families, and workers report failure, be heard, and trigger institutional correction?

This distinction changes the diagnosis. It is possible for a care system to have enough current capacity to keep operating, while lacking sustainable capacity. It can continue only by consuming the time, bodies, patience, and fear of people who have no real exit: disabled older persons, family caregivers, and low-bargaining care workers.

This paper calls that condition forced carrying under unsustainable capacity.

Forced carrying occurs when formal authority and public commitments remain in place, material and execution capacity are insufficient or concealed, and the practical deficit is absorbed by actors who cannot realistically exit. In elderly care, the non-exitable subjects are especially visible. A disabled older person cannot simply opt out of care. A family may be unable to abandon a parent without moral and emotional collapse. A low-income care worker may be unable to refuse unsafe workloads if the alternative is unemployment.

This non-exit condition is not only economic or legal. It is also moral and emotional. The paper calls this mechanism ethical tethering: a person remains attached to a care task because leaving would expose another living, feeling person to immediate harm, humiliation, fear, or abandonment. A daughter may

not be able to leave a parent lying in waste. A care worker may not be able to ignore a resident who has fallen, even when the staffing schedule makes proper care impossible. A nurse or aide may continue compensating for a failed system because the alternative is not an abstract policy failure but a suffering body in front of them.

Ethical tethering becomes institutional exploitation when public systems, families, or providers silently rely on this moral inability to abandon. The hidden calculation is simple: even if staffing is inadequate, even if pay is low, even if time is impossible, caregivers will probably stretch themselves because they cannot bear to watch an older person go hungry, fall, remain soiled, or cry without response. In that moment, a capacity gap is converted into moral extraction. The system does not merely underfund care; it uses the caregiver's conscience as a free buffer.

This also explains why the injury is not only economic. In health-care ethics, related concepts such as moral distress and moral injury describe the damage that occurs when a person knows what care or action is ethically required but is constrained from providing it (Morley et al. 2019), or when a person participates in, witnesses, or fails to prevent events that violate deeply held moral commitments (Litz et al. 2009; Čartolovni et al. 2021). Ethical tethering can produce this kind of injury in care work. The worker is not merely tired. The worker may experience the system as using their conscience against them: they see what should be done, cannot do it under the available conditions, and must still remain present with the person who suffers the consequence.

For Aletheia, this matters because moral injury is a form of subject damage. It attacks the worker's agency, professional identity, and sense of self as a responsible person. A care system that repeatedly produces such injury is not merely inefficient or underfunded; it is consuming the moral integrity of the people through whom it continues to function.

This diagnosis should be separated from three nearby concepts.

First, honest scarcity is not the same as institutional evasion. A public system may truly lack money, staff, or facilities. But if the shortage is real, it should be publicly named: what is missing, who is affected, what standard is being lowered, what risk is being accepted, and what correction path exists. Scarcity becomes domination when it is hidden behind moral language such as family duty, worker devotion, or market choice.

Second, legitimate delegation is not the same as hollow delegation. The public system may delegate care tasks to institutions, communities, families, or market providers. But legitimate delegation requires that authority, resources, training, standards, supervision, and correction travel with the delegated responsibility. Hollow delegation occurs when formal responsibility stays above, practical burden moves below, and the lower actor receives neither capacity nor voice.

Third, increased supply is not the same as sustainable capacity. A system may add beds, agencies, apps, subsidies, or training programs while still relying on overworked labor, unstable staffing, family sacrifice, and underreported harm. Sustainable capacity exists only when the system can reproduce the people, institutions, trust, and resources on which it depends.

The sustainable-capacity test therefore adds a crucial question to the responsibility white-box:

If this arrangement continues for ten or twenty years, who will be worn down, who can leave, who cannot leave, and who is allowed to say that the system is failing?

7.1 Two modes of capacity concealment

Capacity gaps can be hidden in different ways. Two patterns are especially important for comparative analysis.

The first is marketized concealment. A system presents care as a field of choice, quality competition, service innovation, and consumer preference. High-end facilities, polished brands, premium rooms, and private packages make the sector appear dynamic. But if the first-layer floor is insecure, market growth can conceal the fact that life-safety care remains tied to purchasing power, family resources, and labor compression. The visible success of fifth-layer consumption hides the fragility of first-layer and third-layer responsibilities.

The second is familialist concealment. A system presents care as family virtue, filial duty, marital affection, or private morality. The public capacity gap is absorbed inside households, often through the unpaid or underpaid labor of women. The family may indeed be a place of love, but when public institutions rely on love to replace capacity, family morality becomes a fiscal technology. What appears as virtue may be carrying an undisclosed policy deficit.

These two modes can coexist. A society may expand private elder care markets for those who can pay while also expecting families to absorb the needs of those who cannot. In both cases, the diagnostic question is the same: whose need is being recognized, whose power is being protected, and whose body or conscience is being used to carry the deficit?

7.2 Illustrative diagnostic cases: France and England

The French Orpea scandal illustrates marketized concealment. After the publication of Victor Castanet's investigation *Les Fossoyeurs* in 2022, Orpea faced allegations concerning mistreatment, understaffing, rationing, and misuse of public funds (Castanet 2022). French administrative inspections by IGAS and IGF confirmed significant dysfunctions in the management of Orpea nursing homes, and the French state pursued recovery of public funds (IGAS and IGF 2022; French Ministry of Economy 2022). For this paper, the point is not that one company represents all elder care. The diagnostic significance is structural: when a first-layer life-safety need is delivered through a profit-seeking institutional chain, public subsidies and private payments can coexist with execution compression. The system may still look organized at the level of authority and finance, while the pressure is transmitted downward into food, hygiene, staffing time, and resident dignity.

In Aletheia terms, this is a case in which fifth-layer market incentives can squeeze third-layer public operation capacity and damage first-layer life-safety protection. The variable most visibly compressed is E: execution. When execution is cut, the injury does not remain abstract. It appears as fewer hands, shorter care time, delayed response, dirtier bodies, weaker nutrition, and fear among residents who cannot exit.

The English NHS/social care interface illustrates a different but related pattern. Reports on delayed hospital discharge repeatedly point to shortages in social care capacity, including waits for care home beds and home-based care (CQC 2024; King's Fund 2025). Here the problem is not simply a private nursing home scandal. It is a system boundary failure: medical authority, social care capacity, local government finance, labor availability, and family responsibility do not align. Hospitals may be clinically ready to discharge patients, but the receiving care system lacks the capacity to absorb them safely. The result is a backlog that harms hospitals, patients, families, and care workers at once.

In Aletheia terms, the English case shows why C_{actual} is insufficient. A hospital may have the authority and clinical execution capacity to treat an acute episode, but the wider care system may lack sustainable A, M, E, T, and F across the discharge pathway. The person delayed in hospital becomes

evidence of a broken responsibility chain: the need has moved from acute medicine to social care, but the capacity and accountability have not moved with it.

These cases should be read as diagnostic illustrations, not as proof that France and England have the same care system. Their value is that they show two different routes toward the same structural danger: care remains formally promised, but the practical deficit is carried by residents, families, and workers.

7.3 Illustrative diagnostic case: China and familialist concealment

China illustrates the familialist side of the same diagnostic problem. According to the 2025 national bulletin on aging, China had 323.38 million people aged sixty and above by the end of 2025, accounting for 23.0 percent of the population; the same bulletin reports 308.55 million participants in long-term care insurance and 1.93 million benefit recipients (Ministry of Civil Affairs and China National Working Commission on Aging 2026). These figures show both rapid institutional expansion and the scale gap between population aging, insurance enrollment, and actual long-term care receipt.

The diagnostic point is not that China lacks policy activity. On the contrary, long-term care insurance, community elder care facilities, elderly meal services, age-friendly renovation, and public elder-care networks are all expanding. The point is that the public system still operates inside a strong family-care assumption. When formal services are limited, fragmented, or unaffordable, the remaining work is often absorbed by households. This work may appear as filial piety, marital duty, or family affection, but it also functions as a hidden capacity reserve.

Project observation also suggests another edge case: low-cost rural care shelters for older persons outside stable formal coverage. In some small localities, a few adults returning from urban work may open a modest care home for a dozen or so older people whose families cannot provide daily care and whose formal insurance or service coverage is weak. Monthly payments may be only a few hundred to slightly over one thousand yuan. This is not presented here as a national statistic, nor as a criticism of the caregivers, who may themselves be carrying the deficit under severe resource constraints. Its diagnostic value is different: it shows what happens when public responsibility does not reach the person, family capacity is hollowed out, and market price is too low to produce dignified care. What remains is not a profitable market, nor a functioning public system, but survival shelter.

Such cases reveal the difference between keeping a person alive and recognizing a person as a subject. A public system has not fulfilled its responsibility merely because people remain alive, fed, and housed at the lowest possible level. It must still ask whether they remain persons with dignity, memory, relationship, fear, preference, and a claim to more than biological continuation. Survival is not the endpoint of public responsibility; human life, in the fuller sense of dignity and recognition, is.

In NPR-SC terms, the old person's life-safety need is public in its underlying subject, because aging, disability, and dependency are shared vulnerabilities. Yet the practical responsibility can still fall back onto adult children, spouses, daughters-in-law, migrant family members, or hired informal caregivers. Familialist concealment occurs when this residual burden is narrated mainly as virtue while its time cost, income loss, gendered distribution, emotional exhaustion, and lack of exit remain undercounted.

The Chinese case therefore complements the French and English cases. France shows how marketized delivery can compress execution. England shows how a boundary between health care and social care can break the responsibility chain. China shows how an expanding public elder-care system can still rely on household morality as an informal shock absorber. In all three cases, the central question is not whether care institutions exist, but whether the sustainable conditions for care have

been publicly maintained rather than silently carried by those with the least ability to refuse.

8. Dependency-based value inversion in care labor

Care labor presents a sharp value inversion. Society needs care workers because society needs to eat, wash, move, age, fall ill, decline, and die. Yet the workers who handle the most intimate and unavoidable forms of dependence are often described as low-skilled, low-status, or “dirty” labor. The more a task reveals human vulnerability, the more society may try to hide the worker who performs it.

This paper calls the pattern dependency-based value inversion. It is not merely a failure to appreciate labor. It is a reversal produced by denied dependence. Society depends on care workers precisely because they can do what others cannot do, do not want to do, or cannot sustain over time. But in order to avoid acknowledging that dependence, society reclassifies the capacity as low-status, dirty, replaceable, or lacking skill.

Elderly care has a similar structure:

- Society needs care, but wishes not to see dependency.
- Society needs workers who handle excretion, bathing, dementia, immobility, and death, but labels the work as low-end.
- Institutions sell care, families purchase or plead for care, but the worker’s embodied labor disappears into the service package.
- The older person’s dependency is treated as an unfortunate private condition, while the worker’s skill is treated as replaceable.

The inversion is not that care work lacks value. It is that the value is too deep to comfortably admit. To admit dependence on care workers would require society to recognize that its own dignity depends on someone else’s labor. Recognition would require better pay, training, protection, rest, authority, and respect. Devaluation solves the psychological problem by denying the dependence: if the worker is “low-end,” society need not acknowledge that its survival depends on the worker.

This is why elderly care cannot be repaired only by increasing the number of beds or subsidies. It must also repair recognition. Care workers are not merely labor inputs in a service industry. They are carriers of a public operation need. If that is true, then their experience, safety, judgment, schedule, fatigue, and professional development must enter institutional design.

9. Feedback failure: from unmet need to scapegoating

The responsibility white-box also reveals feedback failure. A healthy system should ask whether the original need has been met and whether subject consistency has been restored. In elderly care, negative feedback often takes the form of blame rather than correction.

When families cannot provide enough care, they may be accused of selfishness or lack of filial piety. When older persons complain, they may be described as difficult. When care workers fail, they may be blamed as low-quality labor, even when staffing, pay, training, supervision, and workload made safe care unlikely. When private institutions cut costs, individual workers or managers may be treated as the whole problem, while the financing structure remains unexamined. When public systems are overwhelmed, demographic aging itself may be described as the problem, as if older persons were the cause of the system’s failure.

This is how a responsibility chain becomes a scapegoat chain. Instead of asking which layer of need was misclassified, which authority was over-granted, which responsibility was shifted, and which worker or family member absorbed the cost, the system searches for a morally convenient target.

Aletheia's diagnostic rule is that feedback must return to the subject chain:

- Whose need was the system supposed to serve?
- Did the person receive protection, autonomy, and voice?
- Which public responsibility was transferred to a market, family, or worker?
- Did the practical responsibility-bearer receive authority and resources?
- Did the arrangement produce new domination, invisibility, or dependency?
- What must be corrected upstream?

Without this feedback, care policy risks becoming a cycle of expansion without repair.

10. Institutional repair principles without a complete policy plan

This paper does not offer a complete elderly care policy. It offers institutional repair principles that follow from the diagnostic model. These principles are not a substitute for actuarial design, public budgeting, labor law, or local service planning. They specify what a care system must not hide if it wants to remain morally and institutionally coherent.

10.1 Treat basic disabled care as a life-safety floor

Basic care for disabled older persons should be treated as a first-layer life and safety need. This does not determine the exact benefit formula, financing source, or provider model. It does establish that such care should not depend primarily on whether a family can purchase enough service. A basic floor should be publicly guaranteed, publicly supervised, and publicly correctable.

10.2 Make capacity gaps public

If public capacity is insufficient, the gap should be named rather than displaced. A responsible system should disclose unmet need, waiting lists, staffing ratios, service limits, family burden, worker turnover, institutional losses, safety incidents, and lowered standards. Public honesty does not solve scarcity by itself, but it prevents scarcity from becoming a hidden command imposed on those with the least bargaining power.

This also means distinguishing compliance indicators from capacity indicators. Compliance indicators ask whether an institution has violated rules. Capacity indicators ask whether it has enough money, staff, time, training, retention, and social support to meet the public task sustainably. Many systems are better at the first than the second.

10.3 Treat care worker training and protection as public operation

Care worker supply is not merely a labor market issue. If long-term care is a public operation need, then training, staffing, career paths, occupational safety, rest, professional recognition, and wage floors are part of public capacity. A society cannot declare elderly care important while treating the person who performs it as disposable.

Where fiscal capacity is genuinely limited, non-monetary forms of recognition may matter: predictable rest, safe staffing, training time, mentoring roles, recovery time, public visibility, grievance protection, priority access to learning resources, and better work arrangements. But these are supplements, not

substitutes for the long-term task of improving pay and working conditions. A shortage of money should not be disguised as a moral duty to endure.

10.4 Distinguish family care from forced substitution

Family care can be meaningful, loving, and irreplaceable. But the existence of family affection cannot justify public withdrawal. A good care system should support families without making them the default replacement for missing public infrastructure. The test is whether families can choose care roles without being economically, emotionally, or administratively trapped.

10.5 Make delegation non-hollow

Delegation is acceptable only when capacity travels with responsibility. If public systems purchase services from institutions, rely on communities, contract with market providers, or expect families to participate, then the delegated actor must receive enough authority, resources, information, training, and correction channels to carry the task. Otherwise delegation becomes a way to move failure downward.

10.6 Protect non-exitable subjects

The most important test of a care system is how it treats those who cannot easily leave. Disabled older persons cannot exit their own bodily dependency. Families may not be able to exit moral ties.

Low-wage care workers may not be able to exit unsafe work. A system that balances its budget by consuming these subjects is not efficient; it is transferring cost to the least protected point in the chain.

10.7 Permit high-end elderly services without letting them define the system

High-end elderly services may lawfully exist. They may create comfort, innovation, and variety. The problem begins when the fifth layer expands while the first and third layers remain insecure. Luxury retirement communities should not become evidence that elderly care is solved. Nor should the growth of the “silver economy” obscure unmet life-safety needs.

10.8 Evaluate care by sustainable subject consistency, not only supply volume

Beds, facilities, insurance coverage, expenditure, and market scale are important indicators, but they are not enough. A care system should also be evaluated by whether older persons have voice, whether family burden is reduced rather than hidden, whether care workers receive corresponding resources and authority, whether grievances can be corrected, whether public responsibility remains visible, and whether the arrangement remains sustainable over time.

10.9 What society should do

The social appeal of this paper is deliberately modest. It does not ask society to romanticize sacrifice or demand that ordinary people love care work. It asks for something more basic: stop hiding dependence.

Everyone may age, become disabled, need help washing, need help eating, lose mobility, lose memory, or rely on a stranger’s hands to remain alive. If that is true, then elderly care is not a marginal charity for “other people.” It is a public mirror of shared vulnerability. Society should therefore stop treating care workers as low-end labor, stop treating family exhaustion as private failure, stop treating older persons as burdens or consumers only, and stop treating public capacity shortage as if it can be solved by moral pressure.

The first civic responsibility is recognition: to admit that care is a condition of shared life. The second is disclosure: to demand that institutions name real shortages instead of pushing them onto those who cannot exit. The third is repair: to build feedback channels through which older persons, families, and workers can report failure without becoming scapegoats.

11. Relation to Project Aletheia

This paper extends the general Aletheia thesis that institutions must serve persons rather than absorb them. The first Aletheia working paper, *Human Subjectivity, Public Responsibility, and Institutional Boundaries*, presents the philosophical foundation: persons are ends; needs are fluid; public responsibility arises from shared vulnerability; authority must remain limited, authorized, revisable, and correctable.

The present paper narrows the focus. It asks whether Aletheia's concepts can diagnose a concrete field. Elderly care is an especially strong test because it contains dependency, bodily vulnerability, marketization, family obligation, invisible labor, professional hierarchy, public finance, and moral discomfort all at once.

The result is not a claim that Aletheia has solved elderly care. It is a claim that the framework identifies a failure that ordinary policy language can miss: public care can fail even while supply expands if the wrong layer of need is being served, the wrong actor receives power, and the wrong person bears responsibility.

11.1 Relation to care ethics and recognition theory

This paper does not claim to be the first theory to notice dependency, care labor, or the invisibility of caregivers. It should be read in conversation with at least three nearby traditions.

First, Eva Feder Kittay's work on dependency and dependency labor argues that liberal theories of equality and citizenship often fail to account for human dependency and for the caregivers, often women, who sustain dependent persons (Kittay 1999). Aletheia agrees with this starting point. Its added emphasis is not only that dependency work must be recognized, but that the demand-power-responsibility chain must identify which subject owns the need, which actor receives authority, and where responsibility is displaced.

Second, Joan Tronto's political ethic of care and the idea of privileged irresponsibility show how those who benefit from care can avoid seeing the care relations on which their lives depend (Tronto 1993, 2013). Dependency-based value inversion is close to this tradition. The difference is that Aletheia frames the problem as a diagnostic failure in public responsibility: the powerful are not only failing to care; they are preserving authority while externalizing the labor, cost, and moral burden of care.

Third, Nancy Fraser's distinction between redistribution and recognition helps clarify why care cannot be repaired only through either money or respect alone (Fraser 1995; Fraser and Honneth 2003). Low-paid care work suffers from maldistribution, while the cultural devaluation of bodily, dirty, intimate, and feminized labor involves misrecognition. Aletheia adds a third diagnostic question: whether need, power, and responsibility remain subject-consistent. Redistribution and recognition may both improve, yet a system can still fail if the public need is delegated without sustainable capacity and correction.

The paper's contribution is therefore not to replace care ethics, dependency theory, or recognition theory. It offers a white-box diagnostic method for tracing how a recognized need becomes authority, how authority becomes responsibility, and how responsibility can be displaced onto those least able to

refuse.

12. Conclusion

The crisis of elderly care is not simply the question “who will take care of older people?” It is the question of how a society classifies need, assigns power, and distributes responsibility when every person is vulnerable to age, disability, dependence, and death.

If elderly care is treated mainly as private consumption, those who can pay become consumers, those who cannot pay become burdens, families become residual providers, and workers become low-status instruments of a public function that society refuses to name. If elderly care is treated as a public operation grounded in life-safety needs, the question changes. The society must ask how to guarantee a floor, how to support families without trapping them, how to recognize care workers as carriers of public responsibility, how to allow markets without surrendering public accountability, and how to restore subject consistency when the chain breaks.

The core claim can be stated in one sentence:

Elderly care fails when a public life-safety need remains a formal commitment while the sustainable capacity to meet it is insufficient, concealed, and finally carried by those least able to exit.

A just care system must reconnect these subjects. It must let the need return to the community, the authority return to limited public purpose, the responsibility return to the actor who can actually bear and repair it, and the dignity return to the persons whose vulnerability and labor make the system necessary. The repair is not simply more supply. It is honest disclosure of capacity, protection of non-exitable subjects, recognition of care labor, and institutional feedback that prevents hidden deficits from becoming human endurance.

References

This working paper uses elderly care as a conceptual case. The following sources inform the conceptual framework and illustrative empirical discussion:

- Project Aletheia. 2026. Project Aletheia v0.4.0: Human Subjectivity, Public Responsibility, and Institutional Boundaries. DOI: 10.5281/zenodo.22766372 (<https://doi.org/10.5281/zenodo.22766372>). Repository: <https://github.com/bbbzzzzzzZ/Project-Aletheia> (<https://github.com/bbbzzzzzzZ/Project-Aletheia>).
- Castanet, Victor. 2022. *Les Fossoyeurs*. Fayard.
- Care Quality Commission. 2024. The state of health care and adult social care in England 2023/24, including discussion of discharge delays linked to waits for care home beds and home-based care. <https://www.cqc.org.uk/> (<https://www.cqc.org.uk/>)
- Čartolovni, Anto, Marija Stolt, Riitta Suhonen, and Amir Muzur. 2021. “Moral Injury in Healthcare Professionals: A Scoping Review and Discussion.” *Nursing Ethics* 28(5): 590-602.
- Dean, Wendy, and Simon G. Talbot. 2018. “Physicians Aren’t ‘Burning Out.’ They’re Suffering from Moral Injury.” *STAT*.
- Fraser, Nancy. 1995. “From Redistribution to Recognition? Dilemmas of Justice in a ‘Post-Socialist’ Age.” *New Left Review* 1/212: 68-93.

- Fraser, Nancy, and Axel Honneth. 2003. *Redistribution or Recognition? A Political-Philosophical Exchange*. London: Verso.
- French Ministry of Economy / IGAS / IGF. 2022. Public communication on the IGAS-IGF report concerning Orpea, reporting significant dysfunctions in the organization of the group to the detriment of resident care. <https://presse.economie.gouv.fr/> (<https://presse.economie.gouv.fr/>)
- Inspection générale des finances and Inspection générale des affaires sociales. 2022. Synthèse de la mission sur la gestion des EHPAD du groupe Orpea. <https://www.igf.finances.gouv.fr/> (<https://www.igf.finances.gouv.fr/>)
- Kittay, Eva Feder. 1999. *Love's Labor: Essays on Women, Equality, and Dependency*. New York: Routledge.
- The King's Fund. 2025. Analysis of delayed discharges and the difficulty of measuring NHS/social care discharge delay. <https://www.kingsfund.org.uk/> (<https://www.kingsfund.org.uk/>)
- Le Monde. 2022. Reporting on complaints and public controversy around Orpea and Les Fossoyeurs. <https://www.lemonde.fr/en/> (<https://www.lemonde.fr/en/>)
- Litz, Brett T., Nathan Stein, Eileen Delaney, Leslie Lebowitz, William P. Nash, Caroline Silva, and Shira Maguen. 2009. "Moral Injury and Moral Repair in War Veterans: A Preliminary Model and Intervention Strategy." *Clinical Psychology Review* 29(8): 695-706.
- Ministry of Civil Affairs of the People's Republic of China and China National Working Commission on Aging. 2026. 2025 Annual Bulletin on the Development of China's Aging Undertaking / 2025年度国家老龄事业发展公报. <https://www.cncaprc.gov.cn/> (<https://www.cncaprc.gov.cn/>)
- Morley, Georgina, Natalie Ives, Caroline Bradbury-Jones, and Fiona Irvine. 2019. "What Is 'Moral Distress'? A Narrative Synthesis of the Literature." *Nursing Ethics* 26(3): 646-662.
- OECD. 2023. *Beyond Applause? Improving Working Conditions in Long-Term Care*. <https://www.oecd.org/> (<https://www.oecd.org/>)
- Tronto, Joan C. 1993. *Moral Boundaries: A Political Argument for an Ethic of Care*. New York: Routledge.
- Tronto, Joan C. 2013. *Caring Democracy: Markets, Equality, and Justice*. New York: New York University Press.
- U.S. Government Accountability Office. 2019. *Nursing Homes: Improved Oversight Needed to Better Protect Residents from Abuse (GAO-19-433)*, documenting oversight gaps despite federal quality standards and state survey processes. <https://www.gao.gov/products/gao-19-433> (<https://www.gao.gov/products/gao-19-433>)
- World Health Organization. "Ageing and health — China." <https://www.who.int/> (<https://www.who.int/>)
- State Council Information Office / China government public materials on long-term care insurance and elderly care policy. <https://english.scio.gov.cn/> (<https://english.scio.gov.cn/>) and <https://english.www.gov.cn/> (<https://english.www.gov.cn/>)
- National Bureau of Statistics of China, annual demographic statistics. <https://www.stats.gov.cn/> (<https://www.stats.gov.cn/>)